COUNTRY PROFILE AND MARKET INTELLIGENCE

Singapore

Country profile



AI-generated depiction of Marina Bay Sands and the ArtScience Museum; not a photograph of the site.

Contents

Country overview and government....2
Economy and healthcare outlook....2
Disease prevalence....3
Healthcare delivery....3
Healthcare financing....4
Workforce and education....4
Competitive landscape....5
Institution directory....6
Recent developments....8
CSI assessment....8

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Country overview and government

Indicator	Reported value
Capital	Singapore (city-state)[1]
Official languages	English, Malay, Mandarin and Tamil[2]
Population	6.2085 million total; 4.2315 million residents (June 2026)[3]
Area	745.1 km² (June 2026)[4]
GDP (PPP) total	About Int$998 billion (2025, calculated)[3,5]
GDP (PPP) per capita	Int$163,354 (2025)[5]
Government	Parliamentary democratic republic[1]
Currency	Singapore dollar (SGD)[6]

Singapore had 6.21 million people in June 2026, including 4.23 million residents, a category comprising citizens and permanent residents.[3] Residents aged 65 and older accounted for 19.5% of the resident population, while their share among citizens was 21.4%.[3,7] The resident median age reached 43.6 years, and resident life expectancy at birth was 83.9 years in 2025.[7] The resident fertility rate fell to 0.87 births per woman in 2025, compared with 0.97 in 2024.[7]

Government

Singapore is a parliamentary democracy in which Prime Minister Lawrence Wong leads the government and also holds the finance portfolio.[1,8] President Tharman Shanmugaratnam is the non-partisan head of state.[9] The elected presidency combines ceremonial and community responsibilities with constitutional safeguards over past reserves and specified public-service appointments, rather than day-to-day management of government.[10] For healthcare engagement, the Ministry of Health works with three geographically organized public healthcare clusters that provide services and act as regional health managers, linking clinical delivery with population-health responsibilities.[11]

Economy and healthcare outlook

Economy

Singapore recorded nominal GDP of S$789.5 billion (US$618 billion) and real GDP growth of 5.0% in 2025, according to the latest national accounts; nominal GDP per capita was S$129,194 (US$101,000).[12] On August 11, 2026, the government raised its full-year growth forecast to 4.5% to 5.5% after growth of 6.1% in the first half, citing stronger global investment related to artificial intelligence.[13]

The Singapore Economy 2030 strategy includes advanced manufacturing, with a stated objective of increasing manufacturing value added by 50% from 2020 to 2030.[14] The strategy emphasizes advanced manufacturing investment, skills development and adoption of new technologies, alongside services and business connectivity.[14]

Healthcare outlook

The principal structural demand driver is aging: 19.5% of residents were 65 or older in June 2026, and the citizen share was higher at 21.4%.[3,7] Age Well SG aims to keep older people active, socially connected and supported within their communities, while Healthier SG focuses on preventive care with a regular family doctor.[21,36]

MOH's March 2026 update maintained plans for approximately 2,800 additional public acute and community hospital beds by 2030, alongside new and redeveloped polyclinics.[20] The Health Information Act, enacted on February 3, 2026, establishes a framework for sharing key records and protecting health information across care settings.[37] The official implementation schedule provides for information from general practitioners, private hospitals and specified diagnostic providers from September 2027, followed by specialist outpatient clinics, nursing homes and other designated providers from September 2028.[37] These phased contributions are intended to support coordinated care as more treatment moves into the community.[37]

Disease prevalence

Indicator	Reported value
Leading causes of death	Cancer 26.6%; pneumonia 22.9%; ischemic heart disease 21.2%; stroke/cerebrovascular disease 5.6% (2025 deaths)[15]
Diabetes	9.1% of residents aged 18 to 74 (2023 to 2024, crude prevalence)[16]
Hypertension	33.8% of residents aged 18 to 74 (2023 to 2024, crude prevalence)[16]
Obesity	12.7% of residents aged 18 to 74, BMI ≥30 (2023 to 2024)[16]
Daily smoking	8.8% of residents aged 18 to 74 (2025, crude prevalence)[17]

The mortality profile combines a substantial chronic disease burden with respiratory infection: cancer, pneumonia and ischemic heart disease were the three largest recorded causes of death in 2025.[15] The 2023 to 2024 health examination estimated diabetes prevalence at 9.1%, hypertension at 33.8% and high cholesterol at 30.5% among residents aged 18 to 74.[16] More than half of residents with hypertension in that examination had not previously been diagnosed, reinforcing the value of screening and sustained primary care.[16] The newer NPHS 2025 report covers household interviews, while its next measured chronic disease results are scheduled for the NPHS 2026 report.[17] In 2025, 8.8% reported daily smoking and 87.3% met the survey definition of sufficient total physical activity.[17]

Healthcare delivery

Public healthcare system

Public healthcare is organized into National Healthcare Group in the central region, National University Health System in the west and SingHealth in the east.[11] The clusters combine primary, acute, specialist and intermediate care and work with community and social-service partners as regional health managers.[11] MOH recorded 11 public acute hospitals and 10,784 public acute beds in 2025, alongside a separate psychiatric hospital and public community hospitals.[18,19] By end-February 2026, the polyclinic network had reached 28 sites.[20]

Healthier SG gives citizens and permanent residents aged 40 and older an ongoing relationship with a family doctor and an individualized prevention plan covering lifestyle, screening and vaccination.[21] Government subsidies and the MediSave, MediShield Life and MediFund framework support access to public-sector treatment, with assistance varying by financial circumstances and ward choice.[22] The operating direction is therefore to manage risk earlier and coordinate care across settings rather than rely solely on additional acute beds.[11,21]

Private healthcare system

MOH recorded eight private acute hospitals with 1,649 beds in 2025, compared with 10,784 public acute beds and 334 not-for-profit acute beds.[18,19] Private provision is particularly extensive in primary care, with 2,777 private general-practitioner clinics in the 2025 facilities count.[18] The hospital and professional fee components can be billed separately; MOH publishes recommended private-sector fee ranges to help patients and insurers compare routine cases.[23]

Raffles Hospital is one example of a private hospital maintaining a dedicated international patient center that supports clinical appointments and non-medical arrangements for overseas patients.[24] Singapore regulates healthcare through a services-based framework under the Healthcare Services Act, including newer delivery models such as mobile and virtual care.[25]

Healthcare financing

Indicator	Reported value
Current spending per capita	US$3,921.6 (2023, current dollars)[26]
Government health expenditure	S$17.20 billion (US$13.5 billion); 2.5% of GDP (FY2023)[27]
OECD spending benchmark	9.3% of GDP (2024, total spending)[28]

Singapore combines public subsidy with compulsory medical savings, national catastrophic insurance and a means-tested safety net.[22] MediShield Life provides lifelong coverage for citizens and permanent residents.[29]

WHO expenditure data published through the World Bank place current health spending at US$3,922 per person in 2023.[26] MOH separately reports government health expenditure of S$17.20 billion (US$13.5 billion), or 2.5% of GDP, for FY2023; this fiscal measure includes endowment-fund expenditure, excludes government transfers and is not total national current health expenditure.[27] OECD countries averaged total health spending of 9.3% of GDP in 2024; the different spending scope and reference year prevent a like-for-like comparison with Singapore's government-only figure.[27,28]

Workforce and education

Indicator	Reported value
Doctors	2.9 per 1,000 total population (2024, MOH series)[30]
Nurses and midwives	8.3 per 1,000 total population (2024, MOH series)[30]
Hospital beds	2.83 per 1,000 total population (2025, calculated)[3,19]
Workforce plan	129,000 (2024) to about 156,000 (2030 projection)[31]

MOH reported 17,582 doctors and 50,389 nurses and midwives in 2024, including people not in active practice.[30] Its published ratios were 2.9 doctors and 8.3 nurses or midwives per 1,000 total population.[30] These registration-based series are not directly equivalent to OECD practicing-workforce measures, which averaged 3.9 physicians and 9.2 nurses per 1,000 in 2023 or the nearest available year.[28]

MOH projects the overall healthcare workforce to increase from 129,000 in 2024 to approximately 156,000 in 2030, including growth in community nursing.[31] Hospital capacity in 2025 comprised 12,767 acute, 1,950 psychiatric and 2,579 community beds, or a calculated 2.83 beds per 1,000 total population; nursing-home beds are excluded.[3,19] Definitions and care models should be considered before comparing this figure with the OECD hospital-bed average of 4.2 in 2023 or the nearest available year.[28]

Education

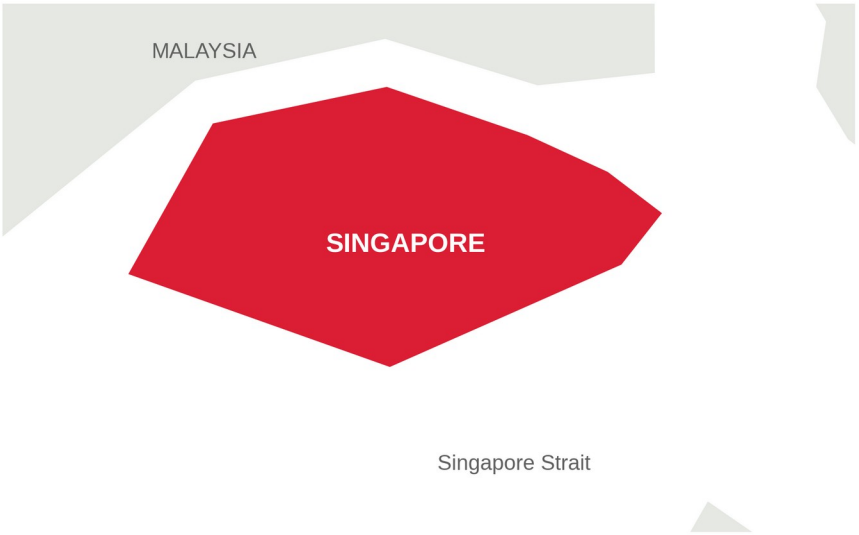
Singapore offers both undergraduate-entry and graduate-entry medical training.[32,33,34] NUS Yong Loo Lin School of Medicine and NTU Lee Kong Chian School of Medicine each provide a five-year MBBS program.[32,33] Duke-NUS offers a four-year graduate-entry MD jointly awarded by Duke University and NUS, with a substantial research and scholarship phase alongside clinical training.[34] NTU now describes its MBBS as an NTU-awarded degree, so historical references to a jointly awarded Imperial degree should not be assumed to describe current admissions.[33]

The Singapore Medical Council permits eligible graduates under provisional registration to undertake 12 months of Postgraduate Year 1 training in approved hospitals.[35] Applications proceed through an accredited local employer, and the route to full or conditional registration depends on the recognized qualification and fulfillment of the council's conditions.[35] NUS includes shared health-professional learning and digital and data literacy in its curriculum, while Duke-NUS integrates population health, patient safety and research experience.[32,34]

Competitive landscape

Singapore's institutional landscape combines three public healthcare clusters with private hospitals serving local and international patients.[11,24] International academic links include the Duke-NUS medical school partnership and a 2025 cardiovascular research MOU between Mayo Clinic and National Heart Centre Singapore.[38,39] These relationships span education, translational research and specialist knowledge exchange.[38,39]

International institutional collaboration



ACTIVE COLLABORATIONS

22

relationship records

21

foreign institutions

National locator
Not facility locations

Active external institutional collaborations, reviewed 7 October 2026. Directory totals exclude Cedars-Sinai. Map: Natural Earth, public domain. Counts describe relationships in the country, not hospital ownership or facility locations.

Singapore also competes for international patients: Raffles Hospital provides clinical appointment support and non-medical assistance for overseas visitors.[24]

Public Cedars-Sinai research collaboration: Guerin Children's participates with Cambridge and Singapore's A*STAR in the BLOOMS neonatal genomics study.[40] A*STAR contributes secure analytics; the announced infant recruitment sites are in the United States and United Kingdom.[40] This is Cedars-Sinai's own institutional research activity, separate from the competing institutions listed here.

Institution directory

Institutions 1 to 12 of 21 with dated evidence of active collaboration. Evidence dates do not establish contract expiration dates.

Institution and location	Nature of collaboration	Evidence
Duke University School of Medicine United States	Research collaboration, Training and education	2025-07[38,41,42]
Harvard University (Harvard T.H. Chan School of Public Health) United States	Research collaboration, Training and education	2026-09[43,44]
Johns Hopkins Health System United States	Training and education	2026-08[45,46]
Mayo Clinic United States	Research collaboration	2025-06[39]
Methodist Le Bonheur Healthcare United States	Training and education	2026-08[47]
Michigan Medicine (University of Michigan) United States	Research collaboration, Training and education	2025-12[48,49,50,51]
Mount Sinai Health System United States	Training and education	2024-11[52,53,54]

Institution and location	Nature of collaboration	Evidence
NYC Health + Hospitals United States	Training and education	2026-08[47]
Northwell Health United States	Training and education	2026-08[47]
Stanford Health Care United States	Training and education	2026-08[45]
Stanford Medicine United States	Research collaboration, Training and education	2024-11[55,56,57,58]
Beijing Tsinghua Changgung Hospital China	Training and education	2026-08[45,59]

Institution directory continued

Institutions 13 to 21 of 21 with dated evidence of active collaboration. Evidence dates do not establish contract expiration dates.

Institution and location	Nature of collaboration	Evidence
CUHK Medical Centre Hong Kong, China	Training and education	2026-08[45,59]
Imperial College London United Kingdom	Training and education	2025-06[60,61]
Karolinska University Hospital Sweden	Training and education	2026-08[45,59,62]
National Cancer Center Japan Japan	Research collaboration	2026-07[63]
Ruijin Hospital (Shanghai Jiao Tong University School of Medicine) China	Research collaboration	2025-10[64]
Shanghai Jiao Tong University School of Medicine China	Training and education	2024-12[65]
University Health Network Canada	Training and education	2026-08[45,59]
University of Cambridge School of Clinical Medicine United Kingdom	Research exchange through joint symposia	2024-11[66,67]
University of Oxford (Oxford University Clinical Research Unit) United Kingdom	Clinical-trial capacity development and training	2025-01[68,69,70,71]

Recent developments

2026-10-03 New national survey reports improved prevention

NPHS 2025 reported chronic-disease screening in 71.5% of eligible residents aged 40 to 74, up from 66.4% in 2024, and sufficient physical activity in 87.3% of residents aged 18 to 74. The screening measure excludes people with previously diagnosed relevant conditions.[17]

2026-09-09 Health regulatory consolidation scheduled in two phases

MOH announced a two-phase consolidation under HSA: MOH regulatory functions and staff are scheduled to transfer in November 2026, followed by the Secretariat of healthcare Professional Boards' functions and staff by end-2027. The professional boards and councils retain their existing self-regulatory and disciplinary roles.[72]

2026-05-06 Health-information sharing moves toward phased implementation

The Health Information Act was enacted on February 3, 2026. MOH subsequently announced progressive record-contribution requirements from September 2027. The current implementation schedule covers general practitioners, private hospitals and specified diagnostic providers first, followed by specialist outpatient clinics, nursing homes and other designated providers from September 2028.[37,73]

2026-03-05 Hospital and primary-care expansion reaffirmed

MOH maintained its plan for about 2,800 additional public acute and community beds by 2030. The update identified the SGH Elective Care Centre for 2027, progressive opening of redeveloped Alexandra Hospital from 2028, and the Eastern General Hospital Campus from 2029.[20]

2026-02-03 Workforce growth planned through 2030

MOH projected growth in the overall healthcare workforce from 129,000 in 2024 to about 156,000 in 2030, factoring in changes in care models and greater demand for community nursing.[31]

CSI assessment

Complete these fields from Cedars-Sinai internal records and human judgment before circulation. Public institutional research relationships do not establish CSI referral activity or commercial terms.

Patient Volume and Revenue

[MANUAL INPUT REQUIRED]

Referral Sources

[MANUAL INPUT REQUIRED]

Relationship History

[MANUAL INPUT REQUIRED]

Complications

[MANUAL INPUT REQUIRED]

Key Contacts

[MANUAL INPUT REQUIRED]

Opportunities

[MANUAL INPUT REQUIRED]

CSI Recommendation

[MANUAL INPUT REQUIRED]